GLICLAZIDE

Preferred SU in elderly

Hypoglycaemia

CHEMICAL NAME N-[(hexahydrocyclopenta[c]pyrrol-2(1H)-yl)carbamoyl]-4-methylbenzenesulfonamide

CLASS Second-generation sulfonylurea

TRADE NAMES Diamicron · Diamicron MR · Glizid · Dianorm

THERAPEUTIC USES / INDICATIONS

- Type 2 diabetes — second-line or when metformin contraindicated; useful in symptomatic hyperglycaemia.

ADMINISTRATION

- **PO** with breakfast (IR: doses >80 mg split twice daily with meals).
- MR tablets once daily with breakfast; swallow whole (may halve 60 mg MR).

DOSAGES

- **IR:** 40–80 mg daily, up to 320 mg/day in divided doses.
- **MR:** 30 mg daily, increase every 4 weeks; max 120 mg/day.
- Elderly: start 30 mg MR.

PHARMACOKINETICS

- **Onset:** 1–2 h. **Duration:** 12–24 h (MR 24 h).
- Metabolised in liver to inactive metabolites.
- Excreted in urine. **t½:** 10–12 h.

PHARMACODYNAMICS

- Closes **β-cell KATP channels** → ↑ insulin secretion; possible antiplatelet/vascular effects.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Weight gain.
- GI disturbance.
- Rash.

ADVERSE EFFECTS (SERIOUS)

- Hypoglycaemia (less than glibenclamide).
- Hepatitis, blood dyscrasias (rare).
- Hyponatraemia.

CONTRAINDICATIONS & CAUTIONS

- Type 1 diabetes; DKA.
- Severe renal or hepatic impairment.
- Sulfonamide hypersensitivity; pregnancy, breastfeeding; porphyria.

KEY INTERACTIONS

- **Miconazole (incl. oral gel):** severe hypoglycaemia; contraindicated.
- **Fluconazole, clarithromycin, alcohol, β-blockers:** ↑ hypoglycaemia risk/masking.
- **Steroids:** ↑ glucose.

NURSING MANAGEMENT

- Check glucose; give with breakfast.
- Withhold if not eating or acutely unwell (seek advice).
- Monitor HbA1c, weight.
- Hypoglycaemia education.

PATIENT EDUCATION

- Take with breakfast; do not skip meals.
- Know hypo symptoms; carry sugar.
- Check glucose before driving (DVLA/traffic rules).

KEY POINTS

- Sulfonylurea with inactive metabolites.
- MR max 120 mg/day.
- Preferred SU in elderly/mild renal impairment.
- Hypoglycaemia; weight gain.